Aciclovir

FORM

Vial containing 250mg or 500mg powder for reconstitution
or 250mg/10ml concentrate for infusion

INDICATION

1.Treatment of Herpes Simplex and Varicella Zoster infections

2. Post exposure prophylaxis for varicella in neonates as per UKHSA Guidance

DOSE RANGE
Indication 1: Treatment

AGE	DOSE	FREQUENCY	ROUTE
0-6 months	20mg/kg	Every 8 hours*	IV

*Reduce dose frequency if creatinine clearance less than 50ml/min (contact pharmacy for advice)

DOSE RANGE
Indication 2: Post exposure prophylaxis

AGE	DOSE	FREQUENCY	ROUTE
0-6 months	20mg/kg	Every 8 hours*	Until Day 21 post delivery**

*Reduce dose frequency if creatinine clearance less than 50ml/min (consult pharmacy for advice)

** For post exposure prophylaxis, conversion to oral aciclovir (see oral monograph) can be considered after a minimum of 48hrs of intravenous therapy.

RECONSTITUTION

Add 10ml water for injection or sodium chloride 0.9% injection to each 250mg powder vial or 20ml to a 500mg powder vial and shake gently until completely dissolved, to produce a 25mg/ml solution.

Concentrate for infusion is already reconstituted to 25mg/ml

DILUTION

Central Administration

The appropriate dose may be given without further dilution (**25mg/ml**) via a syringe pump over 1 hour

Peripheral Administration

Dilute further to **5mg/ml** with sodium chloride as below and infuse the dose over 1 hour

Aciclovir 25mg/ml	8ml
Sodium chloride 0.9%	Up to 40ml total

Gives a **5mg in 1ml** solution. Use the required volume.

METHOD OF ADMINISTRATION

- IV infusion over 1 hour
- **25mg/ml** solution MUST be administered via a CENTRAL LINE

COMPATIBILITY

Solution compatibility	Sodium chloride 0.45%, sodium chloride 0.9%, sodium chloride 0.18% with glucose 10%, sodium chloride 0.45% with glucose 2.5%, sodium chloride 0.9% with glucose 5%, glucose 5%
Solution incompatibility	TPN, lipid
IV Line compatibility (when made to a concentration of 5mg/ml only)	Dexamethasone (sodium phosphate), fluconazole, hydrocortisone sodium succinate, insulin, magnesium sulphate, metronidazole, potassium chloride, heparin The following drugs when **both** prepared in glucose 5% only: milrinone, potassium chloride, sodium bicarbonate, vancomycin No other drugs at the same time in the same line due to high pH
IV Line incompatibility	Caffeine citrate, caspofungin, dobutamine, dopamine, gentamicin, midazolam, morphine, paracetamol, piperacillin with tazobactam.

THIS LIST IS NOT EXHAUSTIVE PLEASE CONTACT PHARMACY FOR FURTHER INFORMATION ON COMPATIBILITY WITH ANY MEDICINES NOT INCLUDED

CAUTIONS, CONTRA-INDICATIONS AND SIDE EFFECTS

If renal impairment develops during treatment, a rapid response normally occurs following hydration of the patient and/or dose reduction or withdrawal. Specific care should be taken in all patients receiving high doses to ensure they are well hydrated, particularly if they have any renal impairment.

FURTHER INFORMATION Extravasation risk due to high pH

STORAGE Store at room temperature

LICENSED STATUS **Not licensed for post exposure prophylaxis**
Dilution in glucose 5% Is an unlicensed use.

APPLICABLE POLICIES West of Scotland Neonatal Guidelines – Immunisation Guideline

UK Health Security Agency Guideline on Post Exposure Prophylaxis for Varicella and Shingles; accessible via www.gov.uk

Consult local policy if applicable

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Administer reconstituted solutions immediately.
All vials, ampoules and infusion bags are for single use only unless otherwise stated.

Dose may vary depending on indication, age, renal function, hepatic function, and concomitant medications.
This monograph should be used in conjunction with the terms of reference document prepared by the West of Scotland Pharmacist Group. Information is correct at the time of publication and as per local practice agreement.
For further advice please contact your clinical pharmacist or pharmacy department